



SRI LAKSHMI DIAGNOSTIC LAB

ICS

Fully Computerized Lab

Mr VENKATESH

Age : 53 Year(s) Male

APL Code : APD-KA-047

Ref Doctor: SELF

Customer :

Sample Type : SERUM

SID : 10920296

Collected on : 2025-10-08 00:00

Regd on : 2025-10-08 15:39

Reported On : 2025-10-08 17:24



CLINICAL BIOCHEMISTRY

Test Description	Result	Units	Biological Reference Ranges
LIPID PROFILE			
Cholesterol - Total (Method: CHOD/PAP)	175	mg/dL	< 200 : Desirable 200-239 : Borderline risk > 240 : High risk
Cholesterol - HDL (Method: Direct)	49	mg/dL	< 40 : Low 40 - 60 : Optimal > 60 : Desirable
Cholesterol - LDL (Method: Homogeneous enzymatic end point assay)	97	mg/dL	< 100 : Normal 100 - 129 : Desirable 130 - 159 : Borderline-High 160 - 189 : High > 190 : Very High
Cholesterol VLDL (Method: Calculation)	28.8	mg/dL	7 - 40
Triglycerides (Method: Lipase / Glycerol Kinase)	144	mg/dL	< 150 : Normal 150-199 : Borderline-High 200-499 : High > 500 : Very High
Total cholesterol/HDL ratio (Method: Calculation)	3.6	Ratio	0 - 5.0
LDL / HDL Ratio (Method: Calculation)	2.0	Ratio	0 - 3.5

Interpretation:

Lipid profile can measure the amount of Total cholesterol's and triglycerides in blood:

Test	Comment
Total cholesterol :	measures all the cholesterol in all the lipoprotein particles
High-density lipoprotein cholesterol (HDL-C):	measures the cholesterol in HDL particles; often called "good cholesterol" because HDL-C takes up excess cholesterol and carries it to the liver for removal.
Low-density lipoprotein cholesterol (LDL-C):	measures the cholesterol in LDL particles; often called "bad cholesterol" because it deposits excess cholesterol in walls of blood vessels, which can contribute to atherosclerosis
Triglycerides :	measures all the triglycerides in all the lipoprotein particles; most is in the very low-density lipoproteins (VLDL).



Lab Technologist

DR. P PRASANNA B.K

MD PATHOLOGIST

Bengaluru - 560100.

P.VENKATESHWARLU

Group Leader

#165/6, Hemagopal Building, Gold Coin Club Road, Singena Agrahara Village, Electronic City Post, Anekal Taluk, Bengaluru - 560100.

NOTE : Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.

Processed at Accin Pathlabs India Pvt Ltd BANGALORE Branch

+91 88257 27935; 88926 98537 | arungmkm@gmail.com

Free Home Sample Collection Lab Timing: 7.00 A.M to 9.00 P.M

Page 1 of 1

Report #



ಸುಶ್ರುತ ಆಯುರ್ವೇದ ವೈದ್ಯಕೀಯ ಕಾಲೇಜು ಮತ್ತು ಆಸ್ಪತ್ರೆ Sushrutha Ayurvedic Medical College & Hospital

(Recognized by NCISM, Ministry of AYUSH, New Delhi, Affiliated to RGUHS, Bengaluru, Karnataka)

KPME Registration : BLU01220AYHL4



LAB REPORT

Name : Mr.VENKATESH
Age : 53Years
Gender : Male
Referred By : Dr.SOWMYA S B



Patient ID : 6348
Report No : SAMC10256350
Bill Date : 08-Oct-2025 7:31 am
Rep Date : 08-Oct-2025 11:51 am

Test Name	Result	Unit	Reference Range
BIOCHEMISTRY			
Fasting Blood Sugar	140	mg/dl	60-110
Method : GZO - FCD COMMENTS : 80 - 99 mg/dL : Normal, 100 - 125 mg/dL : Impaired Fasting Glucose (Pre-Diabetes), >126 mg/dL : Diabetes. reference intervals for FBS from ADA RECOMMENDATION 2015. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes. Impaired fasting glucose (IFG) : Fasting glucose repeatedly is at upper limit, family history or abnormal lipid profile. Advised : HbA1c and clinical correlation. NOTE : In absence of pregnancy, IGT and IFG are risk factors for future DM and cardiovascular disease; they are not clinical entities. A person's blood glucose levels normally move up and down depending on meals, Exercise, sickness, and stress.			
Post Prandial Blood Sugar	380	mg/dl	70-140
Method : GZO - FCD COMMENTS : 140 - 199 mg/dL : Impaired glucose tolerance (Pre-Diabetes), > 200 mg/dL : Diabetes BY ADA RECOMMENDATION 2015 NOTE : A person's blood glucose levels normally move up and down depending on meals, Exercise, sickness, and stress.			
Glycosylated Haemoglobin (HbA1c)	7.22	%	<6.2: Normal 6.0-7.0: Good Control 7.1-8.0: Fair Control >8.0: Poor Control
Method : Nycocard Affinity NOTE : HbA1C gives average Blood Glucose level for the period of 10 - 12 weeks and It need not correlate with Blood sugar levels. Any condition altering Red cell life will alter the GHB values. Low Hb% values may not correlate with GHB. GHB Value should not be taken as a sole criteria for diagnosis. A normal A1C level is below 5.7%. An A1C of 5.7 to 6.4 % indicates prediabetes. People diagnosed with prediabetes may be retested in 1 year. People with an A1C below 5.7% may still be at risk for diabetes, depending on the presence of other characteristics that put them at risk, also known as risk factors. People with an A1C above 6.0% should be considered at very high risk of developing diabetes. A level of 6.5 percent or above means a person has diabetes. Assessed two times per year for patients with type 2 diabetes not on insulin and four times per year for patients with type 1 or 2 diabetes on insulin. Predicts development and progression of diabetic microvascular complications. May rise within one week after rise in blood glucose, but may not fall for 2 to 4 weeks after blood glucose decrease when therapy is resumed. False A1c results may also occur in people with other problems that affected their blood or haemoglobin, such as anemia, heavy bleeding, Iron deficiency, kidney failure & liver disease.			
Mean Blood Glucose	160.51	mg/dl	NIL

----- End Of Report -----

Verified

Authorized Sign